

NHS Devon Integrated Care Board Service Specification

Service Specification No:	
Service:	Annual Physical Health Checks for People with Severe Mental Illness
Commissioner Lead:	Leanne Pate
Provider Lead:	Not applicable
Period:	1 st January 2026 – 31 ST March 2029 (3 months notice will be required to terminate this contract by either party)
Date of Review:	December 2025

1. Population Needs

1.1 National/local context and evidence base

People with severe mental illness (SMI) face stark health inequalities compared to those who do not have mental illness. Whether they are under the care of mental health services or access their care from primary care, they are less likely to have their physical health needs comprehensively met, both in terms of identification of physical health concerns and delivery of the appropriate, timely screening and treatment.

Compared to the general population, people with severe mental illness, such as schizophrenia or bipolar disorder:

- Face a shorter life expectancy by an average of 15–20 years.
- Are three times more likely to smoke.
- Are at double the risk of obesity and diabetes, three times the risk of hypertension and metabolic syndrome, and five times the risk of dyslipidaemia (imbalance of lipids in the bloodstream)

This is due to a number of factors that have grown over a number of years:

- a) A lack of clarity around responsibilities in healthcare provision in primary and secondary care.
- b) Gaps in training among clinicians including nurses.
- c) A lack of confidence and resource across the primary care workforce to deliver physical health checks among people with severe mental illness.
- d) A lack of integration between primary, physical health and mental health services.

Individuals living with severe mental illness are not consistently being offered appropriate or timely physical health assessments despite their higher risk of poor physical health. They are not being supported to use available health information and advice or to take up tests and interventions that reduce the risk of preventable health conditions.

National framework:

NHS Long Term Plan commitment: 390,000 people with SMI to receive full checks annually.

Core20PLUS5: SMI physical health checks are a key clinical area for reducing health inequality.

2. Outcomes

2.1 NHS Outcomes Framework domains & Indicators (please highlight relevant domain(s) and provide details of the applicable measures).

Domain 1	Preventing People from dying prematurely	X
Domain 2	Enhancing quality of life for people with long-term conditions	X
Domain 3	Helping people to recover from episodes of ill health or following injury	
Domain 4	Ensuring people have a positive experience of care	X
Domain 5	Treating and caring for people in safe environment and protecting them from avoidable harm	X

2.2 Locally defined outcomes

Improved physical health monitoring and management for patients with SMI, supporting early identification of comorbidities and promotion of holistic, integrated care.

The primary care Quality and Outcomes Framework (QOF) sets out that 'patients with schizophrenia, bipolar affective disorder and other psychoses and other patients on lithium therapy' should be on the practice severe mental illness (SMI) register.

For these patients, we are required by NHS England to commission good quality physical healthcare provision in primary care that will deliver:

1. Completion of recommended 12 component physical health assessments
2. Follow-up: delivery of or referral to appropriate NICE-recommended interventions
3. Follow-up: personalised care planning, engagement and psychosocial support

3. Scope

3.1 Aims and objectives of service

Objective:

To ensure that patients with severe mental illness (schizophrenia, bipolar disorder, other psychoses, and patients on lithium therapy) receive high-quality, evidence-based physical healthcare in primary care.

Service Aims and Rationale

The purpose of this Local Enhanced Service (LES) is to ensure that adults with Serious Mental Illness (SMI) registered with Devon GP practices receive timely, comprehensive, and high-quality physical health checks and appropriate follow-up interventions.

This LES implements NICE recommended physical health checks for adults with Severe Mental Illness (SMI), complementing core GMS and acknowledging that relevant QOF indicators have been retired. Drug safety monitoring for specialist medicines sits outside the scope of this LES and is provided through separate arrangements.

This service aims to:

- Reduce health inequalities and premature mortality among people with SMI.
- Support the NHS Long Term Plan commitment that at least 390,000 adults with SMI nationally receive a full annual physical health check and appropriate follow-up.
- Improve integration between primary care, community mental health teams, and wider physical health services.
- Ensure alignment with national specifications and data recording requirements.

3.2 Service description/care pathway

Antipsychotic drug safety monitoring is the subject of a separately commissioned local enhanced service (LES): Primary Care Teams should consider any additional blood monitoring required as part of the LES and may wish to undertake this at the same time as the monitoring outlined below to streamline appointments wherever possible.

Service requirements:

1. SMI Register:

Maintain an up-to-date register of all patients meeting the above criteria, in accordance with QOF guidance.

The service needs to provide comprehensive annual physical health monitoring and follow-up for people on the practice SMI register. This includes patients with a diagnosis of:

- Schizophrenia
- Bipolar Disorder
- Psychosis
- Individuals on Lithium regardless of the diagnosis

2. Physical Health Assessments:

Conduct annual, comprehensive physical health assessments covering the nationally recommended 12 components

	PHC Element	How Primary Care Should Undertake This	Clinical Purpose
1	Weight (BMI ± Waist Circumference)	Measure height & weight to calculate BMI. Add waist circumference if metabolic risk suspected (especially central obesity or South Asian ethnicity). Provide weight management advice/referral if BMI elevated.	Detect obesity, metabolic syndrome, medication-related weight gain
2	Blood Pressure (+ Pulse)	Seated BP using validated device. Repeat if elevated. Record systolic/diastolic. Check pulse rate/rhythm due to psychotropic medication risk.	Identify hypertension, cardiovascular risk, medication side effects

3	Blood Lipids / Cholesterol / QRISK	Non-fasting lipid profile acceptable. Calculate QRISK where age appropriate. Discuss CVD prevention incl. statins where indicated.	Cardiovascular disease risk detection
4	Blood Glucose (HbA1c preferred)	HbA1c or fasting glucose. Repeat if borderline. Refer to diabetes prevention/management if abnormal.	Detect diabetes/pre-diabetes; monitor metabolic effects of antipsychotics
5	Alcohol Consumption	Screen using AUDIT-C or similar. Offer brief advice or referral for harmful use.	Identify alcohol-related harm risk
6	Smoking Status	Record smoking status. Offer cessation support, NRT, or referral.	Major modifiable mortality risk
7	Nutrition & Physical Activity	Ask about diet quality, food access, activity levels. Offer exercise referral/social prescribing.	Addresses obesity, CVD, diabetes risk, low wellbeing
8	Illicit Substance / Non-prescribed Drug Use	Sensitive enquiry about drug use (cannabis, cocaine, opioids, etc.). Refer to drug services if required.	Dual diagnosis & physical health risk
9	Medicines Reconciliation / Review	Full medication review incl. psychotropics, side effects, monitoring needs (e.g. lithium, clozapine), adherence.	Prevents medication harm; optimises treatment
10	Offer Follow-Up Interventions	Act on abnormal results: referrals, repeat tests, statins, smoking cessation, weight services, BP management etc.	Ensures checks lead to improved outcomes
11	Access to National Screening	Check uptake of breast, bowel, cervical screening. Provide reasonable adjustments/support to attend.	Addresses cancer inequality in SMI population
12	Sexual Health & Oral Health Enquiry	Ask about sexual health/STI risk, contraception, dental health. Signpost appropriately.	Often overlooked inequality area

^ denotes smoking assessment and offer of treatment components for this patient group via SMOK002 and SMOK005

To note: Alcohol has been retired as an indicator, and although not included in specific indicators, is included with Lipids and glucose being suggested in line with clinical judgement in accordance with the need of each patient. Lifestyle factors such as nutrition and exercise are listed as things to be aware of when considering side effects of medication.

3. Follow-Up Interventions: Deliver, or refer to, appropriate NICE-recommended interventions based on assessment findings.

Follow-Up and Intervention

Where abnormalities are identified, the provider must:

- Offer appropriate lifestyle advice, management, or referral (e.g., smoking cessation, weight management, diabetes care).
 - Provide written feedback and agreed care actions to the patient.
 - Communicate outcomes and actions with the patient's care coordinator or secondary care team (if applicable).
- 4. Personalised Care Planning:**
Provide follow-up care that includes personalised care planning, patient engagement, and psychosocial support.
- 5. Documentation:**
Ensure all assessments, interventions, and care planning are accurately recorded in the patient's electronic health record.

All elements must be recorded using the nationally approved SNOMED codes.

3.4 Invitation and Recall

- Practices must operate a proactive recall and invitation system.
- At least **two invitations** (one written/electronic, one by phone or face-to-face) should be attempted annually.
- Non-responders must be coded appropriately.

3.5 Population covered

All adults aged 18 and over on the practice Serious Mental Illness (SMI) register, including patients with:

- Schizophrenia
- Bipolar disorder
- Other psychotic disorders

This includes individuals managed solely in primary care or jointly with secondary mental health services.

For Devon this equates to circa 10,200 individuals receiving the relevant and appropriate intervention.

3.6 Any acceptance and exclusion criteria and thresholds

- **Patients who decline the check after two documented offers.**

- **Patients currently receiving a full annual health check within secondary care (where the provider shares outcomes with the GP practice).**
- **Those in remission:**

QOF guidance documents contain detail on when clinicians should consider excluding patients from the SMI register because their illness is in remission. National advice states that clinicians should only consider using remission codes if the patient has been in remission for at least five years, that is where there is:

- No record of antipsychotic medication prescribed;
- No mental health in-patient episodes; and
- No secondary or community care mental health follow-up for at least five years.

Practices should review those identified as in remission, to check that they are correctly coded. If they are not truly in remission they will be at higher risk of physical ill health and should therefore be monitored.

Evidence of completion of health checks is not expected for people who have been diagnosed within the last 3 months of the submission.

3.7 Interdependence with other services/providers

When an SMI health check has been completed, with patient consent, the outcomes should be communicated with secondary mental health care for correlation with their own care.

3.8 Provider Responsibilities

Providers must:

- Ensure delivery of all twelve elements within a 12-month period.
- Record data using appropriate SNOMED codes for extraction via GPES.
- Ensure staff are trained and competent in performing physical health checks.
- Ensure timely follow-up and documentation of any abnormal results.
- Maintain accurate records of invitations, attendance, and outcomes.
- Work collaboratively with local community mental health services and ICB-led physical health improvement programmes.

4. Applicable Service Standards

4.1 Applicable national standards i.e. NICE and how they will be assessed.

There are a number of interfacing NICE guidance and standards which are relevant for improving the physical health care of people with SMI. These are set out below.

Identification of risk for or presence of a disease following the recommended assessments should be managed in accordance with the relevant disease-specific NICE clinical guidelines.

A. Mental Health and SMI-specific guidance:

- NICE CG178 / QS80 — *Psychosis and Schizophrenia in Adults*
- NICE CG185 / QS95 — *Bipolar Disorder in Adults*
- NICE NG58 / CG120 — *Coexisting SMI and Substance Misuse*
- NICE PH48 — *Smoking: Acute, Maternity and Mental Health Settings*

B. Cardiometabolic and Lifestyle Risk Management:

- NICE CG189 — *Obesity: Identification and Management*
- NICE NG136 — *Hypertension in Adults*
- NICE NG28 — *Type 2 Diabetes in Adults: Management*
- NICE PH44 / NG90 — *Physical Activity and Exercise*

- NICE CG181 — *Lipid Modification*

C. Substance and Alcohol Use:

- NICE PH45 / QS92 — *Smoking: Harm Reduction*
- NICE PH24 / CG100 / CG115 — *Alcohol Use Disorders*
- NICE CG51 / CG52 — *Drug Misuse: Psychosocial Interventions and Opioid Detoxification*

D. National Framework:

- NHS England (2023). *Physical Health Checks and Follow-Up for People with Severe Mental Illness: Guidance for Integrated Care Systems.*

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4.2 Applicable standards as set out in guidance and/or issued by a competent body e.g. Royal Colleges and how they will be assessed.

4.3 Applicable local standards and how they will be assessed.

4.3.1. Quality Standards and Monitoring

- **Completion target:** ≥60% of eligible SMI register patients to receive a full 6-element health check annually.
- **Follow-up target:** ≥90% of abnormal results to have appropriate clinical action recorded.
- **Equity monitoring:** Practices must monitor uptake by deprivation and ethnicity to identify inequalities.

The ICB may audit practice compliance, data quality, and follow-up rates through quarterly data submissions.

Appendix

‘Physical health check and follow-up interventions for people with severe mental illness - technical guidance’ describes ICB requirements for reporting based on READ, CTv3 and SNOMED codes that should be used to evidence these health checks.

5. Payment Structure

Payment will be made for three activity components per patient, upon completion and accurate coding within the clinical system:

Activity	Description	Payment (£)	Claim Frequency
1. Administrative activity	Identification, recall/invitation, and data recording	£__25.45__	Quarterly
2. Nurse appointment	Delivery of complete annual physical health check	£__25.45__	Quarterly
3. GP review	Clinical review, interpretation, communication with patient and/or CMHT	£__38.17__	Quarterly

Total per completed health check: £89.07

Payment will be £89.07 per patient and broken down into three parts in recognition that this cohort of patients may be difficult to engage in the whole process:

- a) A baseline payment of £25.45 to be made for all patients on the SMI register to cover administration of searching and inviting patients to their annual health check.
This should be claimed on a quarterly basis as
 - number on register in the quarter x £6.36 (equating to £25.45 per annum).
 - Severe mental illness enhanced services administration
 - Version 3: XaZWY
 - Version 2: 9ku..
 - SNOMED: 843651000000108
- b) A payment of £25.45 for nurse/HCA appointments. completed in the quarter.
- c) A final payment of £38.17 for GP review and completion of the proforma evidencing that the patient has been offered the opportunity to discuss all the components of the health check. For those completed in the quarter.

Payment rate will be reviewed annually aligned to associated national guidance.

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

6. Reporting:

Reporting is via the commissioning portal quarterly - within a month of the end of the quarter - and include data from each patient over the previous 12 months.

Indicators and calculations

The standard:

Denominator: The ICB has to report to NHS England the total number of people on the General Practice SMI registers (on the last day of the reporting period) excluding patients recorded as 'in remission'.

Numerator: Out of the denominator, the number of people who have received a comprehensive physical health assessment (i.e. all of the checks 1-12 above) in the 12 months to the end of the reporting period, delivered in a primary care setting.

Note that a person who has received all elements of the physical health check would be reported in all of the individual numerators.

It is recognised that people will have been on the GP SMI register for different durations and that some people may have had limited opportunity to be offered physical health checks in primary care; this is considered an acceptable limitation of the data collection.

Calculations: Utilising the numerator and denominator definitions above, the percentage of people receiving health checks will be calculated as:

$$\% = 100 * \text{Numerator} / \text{Denominator}$$

Monitoring DNAs

Other long-term condition QOF registers will pick up elements e.g Asthma, COPD, Diabetes re Lifestyle indicators if our patients have a prior diagnosis. People with SMI are less likely to engage with their health checks in general and may need additional encouragement. However there should be extra vigilance to ensure that frequent DNAs are monitored and not just exempted. In line with QOF SMI requirements, patients should be invited a minimum of two times in the year.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- a. suspension of some or all elements of it,
- b. redirection of patients to alternative provider(s),
- c. in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider

7. . Applicable quality requirements and CQUIN goals (Optional use)

5.1 Applicable Quality Requirements (for inclusion Schedule 4A-C)

Not applicable

5.2 Applicable CQUIN goals (see Schedule 4D) *Generally CQUIN schemes are determined at contract level rather than specification level*

Not applicable

8. Location of Provider Premises (Optional use)

The GP provider's registered premises.